

CLAMIDE TABLET

DESCRIPTION

Oblong, white uncoated tablet, bevel-edged, flat faces, "HD" embossed and scored on the same face

COMPOSITION

Each tablet contains: Glibenclamide 5 mg

PHARMACODYNAMICS

Glibenclamide is an oral hypoglycaemia agent with actions similar to those of chlorpropamide. It promotes the increased secretion of insulin from the beta cells of islet tissue in the pancreas by means of a process not yet specifically defined. It also decreases hepatic glycogenolysis and gluconeogenesis. Apparently, glibenclamide increase the insulin sensitivity of extrapancreatic tissues.

The overall effect is a reduction in blood glucose concentration only in those patients whose pancreas is capable of synthesizing insulin. Oral antidiabetic medications apparently do not influence the production of insulin by the beta cells but seem to enhance its release from these pancreatic cells.

PHARMACOKINETICS

Absorption: About 45% of a 5 mg dose is absorbed after oral administration

Blood Concentration: After an oral dose of 5 mg, peak serum concentration of about 0.05 ug/ml are attained in 2 to 4 hours.

Half-life: Plasma half-life, 3 to 7 hours; half-life determined from urinary excretion data, about 10 hours.

Distribution: Widely distributed throughout the body and does not appear to accumulate

Protein binding: About 98% bound to plasma proteins.

Metabolic reactions: Hydroxylation of the cyclohexyl ring at positions 3 and 4 ; in the rat, rabbit and dog the 3-cis hydroxy metabolite is formed and in the rat the 4-trans-hydroxy metabolite is additionally produced; hydroxylated metabolites may be conjugated

Excretion: In 5 days, 95% of a dose is excreted in the urine and faeces with about 54% excreted as unchanged drug; of the 95% excreted about 75% is found in the faeces and 20% in the urine; excretion of an intravenous dose is more rapid than that from either oral or intramuscular doses.

INDICATIONS

For treatment of mild or moderately severe uncomplicated maturity-onset diabetes mellitus unresponsive to diet alone.

CONTRAINDICATIONS

Not recommended for patients with burns, diabetic coma, severe infection, Type 1 diabetes mellitus, ketosis, ketoacidosis, major surgery, severe trauma, impaired renal, hepatic, thyroid or adrenal function, pregnancy and lactation.

WARNING AND PRECAUTIONS

- Glucose urine tests and ketone tests should be monitored.
- To be taken in conjunction with meals.
- Obese patients should avoid sulphonylureas unless symptoms and diabetic control have not improved despite weight loss to within 1.5%.
- Clamide Tablet is not recommended for children.

PREGNANCY AND LACTATION

Not recommended in pregnancy and lactation.

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INTERACTIONS WITH OTHER MEDICAMENTS AND OTHER FORMS OF INTERACTION

- Concurrent use with monoamine oxidase inhibitors, oral anticoagulants, oxyphenbutazone, phenylbutazone and alcohol may increase glibenclamide activity.
- Blood glucose levels may be increased when used concurrently with corticosteroids, epinephrine, phenytoin, thiazide diuretics and thyroid hormones.
- Co administration of beta adrenergic blocking agents may mask symptoms of developing hypoglycaemia thus complicating patient monitoring.
- An increased hypoglycaemic effect has occurred or might be expected with ACE inhibitors, alcohol, allopurinol, some analgesics (notably azapropazone, phenylbutazone, and the salicylates), azole antifungals (fluconazole, ketoconazole, and miconazole), chloramphenicol, cimetidine, clofibrate and related compounds, coumarin anticoagulants, halofenate, heparin, MAOIs, octreotide (although this may also produce hyperglycaemia), ranitidine, sulphinpyrazone, sulphonamides (including co-trimoxazole), tetracyclines, tricyclic antidepressants, and thyroid hormones.

MAIN SIDE / ADVERSE EFFECTS

- Nausea, vomiting, epigastric pain, dizziness, weakness, paraesthesia and sensitivity reactions.
- Prolonged hypoglycaemia has been reported following the ingestion of glibenclamide.
- Cross sensitivity to sulphonamides or their derivatives may occur. Transient visual disturbances may occur at the start of treatment. Reversible leucopenia and thrombocytopenia have been reported but are rare. Agranulocytosis, pancytopenia and haemolytic anaemia have been reported very rarely.
- Treatment with sulphonylureas has been associated with occasional disturbances of liver function and cholestatic jaundice. If hepatitis or cholestatic jaundice occurs, glibenclamide should be discontinued.

OVERDOSE

Clinical features: Nausea, vomiting, abdominal pain, drowsiness, coma, twitching, convulsions, hyperpnoea, sinus tachycardia, hypotension, hypoglycaemia and metabolic acidosis.

Treatment: Treat overdosage by emesis or gastric lavage if appropriate and correct hypoglycaemia, if any, with 50 ml of 50% glucose IV repeated as necessary and / or glucagon 1 - 2 mg IV, followed by an IV infusion of 5 - 10% dextrose for 24 - 72 hours as necessary.

DOSAGE AND ADMINISTRATION

Oral, initially 5 mg daily, the dosage then being adjusted as needed with maximum daily dose of 15 mg if necessary.

The information given here is limited. For further information, consult your doctor or pharmacist.

Storage: Store below 30°C. Protect from moisture.

Presentation / packing:
Blister pack of 100's

Product Registration Holder: HOVID Bhd.
121, Jalan Tunku Abdul Rahman (Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Manufactured by: HOVID Bhd.
Lot 56442, 7 ½ Miles, Jalan Ipoh / Chemor,
31200 Chemor, Perak, Malaysia.

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